



Assessment and Diagnosis

ASSESSMENT: PARTS VS. PATHOLOGY

Mental health assessment is typically organized around diagnosing pathology, which is designated according to lists of symptoms chosen by committees of professionals: bipolar disorder, schizophrenia, schizoaffective disorder, depression, PTSD, anxiety, OCD, eating disorder, addiction and a host of “personality” disorders (American Psychiatric Association, 2013). The IFS understanding of the psyche orients our assessment of a client’s functioning and potential in a different direction. The mind is plural so we assess the activities of a plural mind. At the outset of therapy we hear the client’s presenting complaint as one part, we then ask permission to talk to any other parts who are involved, we ask them to unblend (separate, differentiate), which makes room for the client’s Self so we can explore how they serve her (if they are protectors) or (later in therapy) how they are burdened and what they need (if they are exiles).

FIRST, IS YOUR CLIENT SAFE?

Before we do anything, however, we want to know if the client is currently safe. If not, we attend to safety first. Traumatized individuals who seek therapy may lack the economic and social resources to be safe for any number of reasons. Physical safety (food, shelter and safety from violence) of course provides a platform for internal inquiry. This is not to say that we can't do parts work if a client is at risk of being undernourished, without shelter, without medical care or is in a dangerous relationship.

But in this circumstance we reach for appropriate resources (domestic violence expertise, a safe house, sources of food and medical care, etc.) to build external safety while establishing rapport in the internal system by noticing parts and setting the intention to help them. Here is an example of a client describing an incident of partner violence at home.

JOSIE: We got into an argument and he grabbed me by my hair and pulled me around the bedroom.

THERAPIST: Has this ever happened before?

JOSIE: Once or twice.

THERAPIST: What is this like for you?

JOSIE: I'm divided. My heart is jumping out of my chest and I'm afraid of how far he might go. But at the same time I feel so enraged that I keep screaming at him.

THERAPIST: What do you say?

JOSIE: Oh "be a man" and other things that are guaranteed to enrage him. And I hit him. I know it's not smart but I don't feel like I can stop.

THERAPIST: Is alcohol a factor?

JOSIE: Probably. I don't think it's ever happened if we haven't been drinking.

THERAPIST: Okay. Can I make some suggestions?

JOSIE: Yes please.

THERAPIST: So let's talk about your safety right now, when you go home. First, is this over? Let's talk now about what usually happens between you after you get violent. Let's make a plan for your safety. And I will give you the number of the domestic violence hotline so you can run the plan by them before you go home. Then - and we may need to do this next week - I want to check in with all parts of you who are involved with this so we can find out who feels what and who thinks what. How does that sound?

JOSIE: That's good. I can feel that - like my heart is a little calmer.



After this conversation the client and therapist can establish guidelines for: a) avoiding further violence, b) de-escalating, if possible, and c) putting other supports in place, like a connection with a domestic violence shelter. De-escalating may be beyond the client's reach in heated moments until she has become more Self-led, and her partner may in any case have demonstrated an unwillingness or inability to deescalate. Our long-term goal is for her protectors to unblend so her Self can take the lead externally as well as internally, but this takes time. After addressing practical matters of safety, we help the client engage her internal system inclusively, without judging or exiling any part, including the ones who are attached to a violent partner, the ones who feel enraged about that, and parts who have varying opinions about what should be done both short and long term.

DOING THE INITIAL ASSESSMENT IN IFS

At the outset of therapy, before we know the client, we check on his access to Self-energy by being curious about his inner experience: How spacious is it? At what speed is it going? How weighty does it feel? How light or dark is it inside? How calm or agitated? For us, these states of being – when not governed entirely by biology – are secondary to inner relationships. The plural model of mind guides us to be curious about the instrumental nature of the client's inner relationships. How does he treat himself? Can he be kind to himself or is he hounded by self-

criticism or constantly on the run? If so, why? How is self-criticism protective? What function does avoidance serve within his system? We trust that his symptomatic behavior will make sense once we know him well enough to see it in the context of formative experiences.

In IFS, Assessment Never Ends

Since our overarching goal in IFS is to improve access to the Self, our emotional and intellectual compass, we start by assessing the level of Self-energy in both client and therapist and we continue to do this throughout therapy. For the therapist, assessing Self-energy means, first and foremost, being aware of our parts as they get activated in response to the client's parts. Secondly, we work with our triggered parts and encourage them to step back so they won't get in the way.

Do We Need Information about the Past?

In an initial assessment we listen to whatever story is relevant to the client's presenting problem, we are curious about history, and we assume that, at some point during therapy, her observations will track back to lessons from the past about danger and safety. Although this trek will be central to her process, we assume that we are not hearing the whole story at the outset of therapy. In addition, we don't need this information to do our job. We can help the client regardless of what she reports to us about historical events.

MEDICAL MODEL VS. MODEL OF MIND

In addition to posttraumatic stress disorder (PTSD) and dissociative identity disorder (DID), trauma survivors often receive a slew of other diagnoses including depression, anxiety, borderline personality disorder, or some kind of addiction to alcohol, drugs, exercise or food (Herman, 1997; Herman & van der Kolk, 1989). These diagnoses are listed in the *Diagnostic and Statistical Manual of Mental Disorders* or DSM (the latest version of which is the DSM-5[®]). The American Psychiatric Association (APA) developed the DSM to ground psychiatric treatment in science. Given their methodology – DSM diagnoses are based on lists of symptoms that are periodically chosen by committees of professionals who are picked by the American Psychiatric Association – this effort remains controversial (Greenberg, 2013; Fisher, 2014). Nevertheless, because the terms of the DSM continue to be widely used, the medical model approach continues to be a strong influence on the field of mental health.

A DIFFERENT APPROACH WITH TRAUMA

In IFS we simply see DSM diagnoses as various ways of describing the behaviors of activated parts. And rather than pathologizing symptomatic behaviors we view them as natural efforts to solve problems: to cope, stay safe and survive. Trauma diagnoses include posttraumatic stress disorder (PTSD), dissociative identity disorder (DID) (van der Kolk, 2011), both listed in the DSM, and complex trauma or developmental trauma disorder (D’Andrea et al, 2012; van der Kolk, 2005; van der Kolk, 2014), which is also widely used, including by the International Society for Traumatic Stress Studies (ISTSS).

Here is an IFS perspective on several diagnoses that are often given to traumatized individuals before (or in lieu of, or in addition to) receiving one of the more specifically trauma related diagnoses listed above.

- **Borderline personality disorder (BPD):** This diagnosis offers a portrait of the consecutive blending of exiles (desperate young parts, internally shunned, longing for rescue and redemption) and protectors, most notoriously (though not exclusively) the ones who forbid the risks of intimacy and the ones who believe that dying is the only way to end emotional pain.
- **Narcissistic personality disorder (NPD):** This diagnosis shows the efforts of a hard-working protector as it holds up a gilded self-portrait as a shield against the arrows of shaming – mostly inner shaming in response to feelings of inadequacy.
- **Depression:** Mood disorders are heritable but not all post-trauma depression is evidence of a genetically-based mood disorder. Since depression suppresses the body’s emotional signals, deadening physical and emotional experiencing in a paralyzing (if excruciating) way, a protector who amplifies depression generally aims to inhibit while an exile who feels depressed is being inhibited.
 - To assess the client’s situation, we ask: “Is this a part of you who feels depressed (exile) or a protective part who is using or magnifying depression for a reason?”

The only way to discover the function of any part you encounter is to ask.
- **Anxiety:** As temperament research indicates (Kagan, 2010), our genes can also make us vulnerable to anxiety. And, as with depression, protectors can push this lever to exert influence.

- “Is this a part (an exile) who feels anxious or a part (a protector) who has some reason to magnify anxiety?” Many protective parts are rooted in fear and carry some percentage of anxiety within them. Again, to find out we ask.
- **Obsessive-compulsive disorder:** OCD behaviors are generally geared toward managing anxiety. In the case of trauma, the preoccupation or behavioral repetition of OCD serves to distract from emotional pain.
 - As with depression and anxiety, to find out how a behavior serves or what story it can tell, we must ask.
- **Sociopathy:** Unless sociopathy is a product of brain damage, it is a protective part (Schwartz, 2016). With their telescopic focus and determination to suppress inner vulnerability, sociopathic protectors are paranoid, extreme and rejecting of both empathy and compassion as weakening. While they protect exiles who they view as unbearably impotent and tender, they are often polarized with other protectors whom they consider weak or caretaking. On this topic, Schwartz wrote:
 - “A perpetrator part can be thoroughly blended all the time, in which case the individual is likely to meet criteria for a DSM-5 diagnosis of antisocial personality disorder. When a perpetrator part stays blended all the time in this way and the client has no access to other parts, we consider it a manager rather than a firefighter.” (2016, p.113)

Addictive disorders:

- **Drugs or alcohol:** The reactive protector who uses a drug or alcohol to distract from emotional pain can also settle into being a proactive protector who uses them to avoid feeling anything at all. This addict part is not a lone actor but is an actor in an inner dynamic.
 - Cykes (2016) elucidates the IFS perspective: “Rather than defining addiction as the behavior of one acting-out part, I define it as a systemic, cyclical process that is characterized by a power struggle between two teams of protective parts, each valiantly struggling to maintain a balanced inner system. One team is critical and judging, the other impulsive and compulsive. Their chronic, escalating struggle is intended to block... emotional pain.” (p. 47)

- **Eating disorders:** ED's illustrate a protector polarity, with excess on one side and inhibition on the other.

- **Bulimia** illustrates both sides of this polarity.
- **Anorexia** illustrates inhibition in the driver's seat.
- **Binge eating** illustrates disinhibition in the driver's seat.
- **Over-exercising** illustrates inhibition in the driver's seat. Catanzaro (2016) describes the IFS lens on ED phenomena: "ED protectors always polarize into two camps: parts who push for restriction and control of the body and parts who reject this control and push for less restraint. Their tug-of-war keeps the client from being aware of the intense negative feelings and memories of exiled parts. While an individual's specific ED diagnosis depends on which parts dominate at any given time, the overall symptom picture, even if it isn't obvious from the client's physical appearance or self-report, always involves this dialectic between restraint and rebellion against restraint." (p. 51)

PRIORITIZING PROCESS OVER CONTENT IN THE FIRST STEPS

Until we reach one of the final steps described later as "the unburdening process," we prioritize process over content – we want to know about inner relationships, agendas and beliefs. This is very different from an approach that seeks to come to a conclusion about content (the client's symptoms) early on in therapy. The following three scenarios illustrate how an IFS session might unfold in response to a client wanting to talk about diagnosis.

CLIENT'S VIEW OF DIAGNOSIS

Scenario #1

SIMON: I'm wondering if I have schizophrenia.

THERAPIST: Let me be sure I understand. You have a part who wonders if I think you have schizophrenia?

SIMON: Yes. It wonders if you think that.

THERAPIST: Do any other parts want to be in on this conversation?

SIMON: I'll ask.

THERAPIST: Do you notice anyone else?

SIMON: I do have one part who doesn't want to hear all this. It's mad that I brought it up. See, I keep hearing that I'm mentally ill, which makes me want to drink.

Scenario #2

THERAPIST: Let me be sure I understand. You have a part who wonders if I think you have schizophrenia?

SIMON: Not exactly. It wants to know if you have enough evidence to say that I'm schizophrenic.

THERAPIST: Would it be willing to say more?

- *Always a good option when we're not sure what to do next.*

SIMON: It thinks if I had a diagnosis I could get better.

THERAPIST: Are there parts who disagree?

Scenario #3

THERAPIST: Let me be sure I understand. You have a part who wonders if I think you have schizophrenia?

SIMON: There's a part who wonders whether to trust you.

THERAPIST: So you have a part who wants to know if I'm trustworthy?

SIMON: Yes.

THERAPIST: Does this part have a view about your diagnosis?

SIMON: Yes.

THERAPIST: Would it be willing to share?

SIMON: If you say I have schizophrenia this part won't trust you.



As these three scenarios illustrate, when we are curious about the client's view of diagnosis we are able to learn about the client's inner discussions and avoid getting inducted to one side of an overheated disagreement (what we call a "polarity").

ASSESSMENT AND DIAGNOSIS IN IFS

In IFS we welcome the client's symptoms upfront as an introduction to protective parts and we are always aware that wounded parts have been banished. To assess and diagnose we look at relationships and motives in the internal system. Regardless of the client's symptoms, we make certain assumptions about a predictable, commonly shared psychic structure in which protectors aim to hide the existence of emotionally vulnerable parts and guard them from being hurt again. Finally, despite protection that is focused on past and future injury, the Self is always available to heal old wounds and take the lead in a dangerous world.

Although we can translate our parts-based observations of the client into DSM diagnoses in order to communicate with other treaters and to bill insurance, in IFS we do not formulate the client's presenting problem in the pathologizing terms of the DSM. Rather, we explore inner relationships, inquire about motivation and ask about protector fears so we can discover how this particular client's inner system maps onto our basic template of the psyche (multiplicity consisting of parts and Self, and post-trauma coping that involves protectors and exiles).

Even as we assess the client's reasons for coming to therapy and her presenting problems, we begin to form relationships with her internal system by asserting the positive intentions of her protective parts and offering to introduce them to her Self. When appropriate, we also offer her a roadmap of our goals:

- No part of you needs to be exiled or sacrificed.
- You will have the opportunity to offer a new solution to the problems your hardworking inner system is trying to solve with these behaviors.
- If your parts take you up on this offer, their emotional pain will eventually be healed and they will feel freer.

PSYCHOLOGY AND BIOLOGY

While we assess the relationships and protective activities of the client's system, we also assume symptomatic behavior may derive from protective parts using biological vulnerabilities (temperament, genetics, somatics) to gain influence. Most mental health problems have both psychological and biological components. In IFS we are interested in teasing out the portion of the problem that is produced by motivated parts and the portion that is rooted in biology. We can only do this by interviewing parts. Since assumptions and generalizations may be (and often are) wrong, we rely on questions.

For example, one person may have a part who feels depressed in response to being relentlessly shamed (an exile) by an inner critic, another person may have a part who amplifies symptoms of depression to make him stay home and avoid taking risks (a protector), while another person's depression may be primarily biological. To find out who is doing what to whom and for what reason we must ask. In the example below, a client reports that he has long had chest pains with no discernible medical origin.

HEALING SOMATIC SYMPTOMS

Find

JAY: I don't feel anything

THERAPIST: Say more.

JAY: Well, when you, or anyone else for that matter – including my wife or my kids – ask me what I'm feeling I just don't know how to respond. I don't seem to feel things the way most people do.

THERAPIST: Do you want to explore this?

- *Asking for permission.*

JAY: Yeah.

THERAPIST: Would it be okay to close your eyes? Good. Now notice any thoughts, feelings or physical sensations that come up.

- *Starting with the body.*

JAY: Okay.

- *After a few seconds, Jay opens his eyes.*

JAY (continuing): I'm aware of chest pains.

Focus

THERAPIST: Is it okay to focus on those to see what we can learn?

- *Asking for permission.*

JAY: I've had chest pains for years. That's nothing new. I've been to several doctors and they keep saying nothing is wrong. Once I even thought I was having a heart attack. I called an ambulance and went to the ER. Still nothing was wrong with me.

- *Jay is completely unaware of any psychological reason for this pain, which speaks to the high level of protection that somatizing often represents.*

THERAPIST: Would it be okay to go back inside and be curious about the chest pain?

I believe all physical sensations hold important information for us.

- *Asking for permission again.*

Flesh Out

JAY: Okay I'll try.

- *After a few moments.*

JAY (continuing): I saw myself as a boy. He must be 8 years old.

- *A part is beginning to show Jay about his experiences, we call this “witnessing.”*

Therapist: What did you see?

JAY: The day my grandfather died.

THERAPIST: You look puzzled.

JAY: I am.

Befriend

THERAPIST: Are you open to seeing more?

JAY: Yes.

- *Jay closes his eyes and is quiet. Tears well up.*

JAY (continuing): Grandpa was very important to me. My father left when I was 4 years old so he was my dad.

Therapist: Is the chest pain connected?

JAY: Yeah. I don't know how.

THERAPIST: Are you open to hearing more?

JAY: I'm really curious now.

THERAPIST: Send your curiosity to the chest pain and ask what it wants you to know.

JAY: This sounds strange but I'm getting that the chest pain helps me not to have feelings.

- *The chest pain is a protective part.*

Assess Protector Fears

THERAPIST: Ask it to say more about why it's important not to have feelings.

JAY: I'm seeing this fort I made in my bedroom. It was a blanket cave under a couple of chairs. I used to go in there when I was upset.

THERAPIST: Does this make sense to you?

- *Checking on Jay's Self-energy toward the boy.*

JAY: Totally.

THERAPIST: Let him know that you're getting the connection.

Witness the Exile's Experience

JAY: I grew up in a family that was not into feelings. When grandpa died, my mother had a birthday party for my older brother the next day. She baked a cake, wrapped the presents and set the table. Her attitude was: the show goes on.

- *Jay is now noticing the extremity of the boy's experience and he is speaking in the first person, indicating that he sees this experience from the perspective of the boy. Even so, this is not what we call a "blended" experience because the boy feels Jay is there with him, he is confidently showing Jay's Self about his experience.*

THERAPIST: What was that like for him?

JAY: Confusing. I knew it was crazy to have a party right after grandpa died. I felt like the world had ended. My brother was miserable. I'm getting that the chest pain helped me get through that time so I did not feel those feelings. It was a big distraction.

- *The chest pain is a part. Technically, we would consider the chest pain a protective part of the boy: a subpart (parts have parts). But because the boy and Jay's Self are in synch we can effectively address the chest pain as simply a part.*

THERAPIST: How do you feel toward the chest pain now?

- *Checking on Jay's level of Self-energy for this protector.*

JAY: I appreciate what it did for me. It really likes that I am getting this.

THERAPIST: I'm wondering if the chest pain would be interested in not having to work so hard?

- *This is our invitation to try something new.*

JAY: I do sense it's exhausted.

THERAPIST: If it will let us help the boy with those feelings, we can get him out of the past.

JAY: The boy wants that.

THERAPIST: So let's ask the chest pain if it will let you help the boy.

- *We always ask protective parts for permission. If we forget, they will jump in anyway.*

Witnessing the Exile's Experience Again

JAY: Okay. I can see him in the fort in my bedroom.

THERAPIST: How do you feel toward him?

- *Checking on Jay's Self-energy.*

JAY: I feel a lot of love for him right now.

THERAPIST: Is he receiving your love?

- *Checking on the connection between the boy and Jay's Self.*

JAY: Wow, he's looking up at me and smiling.

THERAPIST: Great. What does he want you to know?

JAY: He seems really sad.

THERAPIST: Is this level of feeling okay for you?

- *Checking on the reactivity of Jay's protectors.*

JAY: It's a bit intense.

THERAPIST: Let him know that we're here to help him with his sadness. And ask him to share it with you a little at a time so you don't get overwhelmed.

- *Negotiating exile overwhelm.*

JAY: He's okay with that.

THERAPIST: Good. Just be with him and let him share what he wants you to know.

- *Jay sits quietly for several minutes, tears streaming down his face as he witnesses the boy's grief.*

JAY: He is so sad. He's sad about my father leaving, sad about grandpa dying and sad that Mom couldn't handle feelings.

THERAPIST: Does that make sense to you?

JAY: Totally.

THERAPIST: Is there anything else he wants to share?

JAY: I think that's it.

- *When a client uses the word "think" be aware that a thinking part may have stepped in – so we ask him to ask the part for an answer directly.*

THERAPIST: Ask him.

JAY: He says that the chest pain started when grandpa died because that's when I was really alone, no father, no mother and now no grandpa.

THERAPIST: Wow. What does he need from you?

JAY: Just this.

- *We are silent until Jay shifts in his chair.*

THERAPIST: Is he ready to leave that time? He can come to the present with you or go somewhere else that's safe.

JAY: He wants to be with me.

THERAPIST: Okay. Are you in the bedroom with him?

JAY: I'm picking him up.

THERAPIST: Great. Bring him to the present.

- *This is called "retrieval." But it doesn't seem to be working because Jay is now frowning.*

THERAPIST (continued): What's happening?

JAY: I think some other part doesn't want me to take him.

THERAPIST: Ask why.

JAY: I used to get silent a lot when I was a kid. It feels like that part.

THERAPIST: Would the silent part like to come too? (Jay nods.) Great. Bring it up to the present with the boy but ask it to give you a few more moments with him. We can help it afterward.

- *The silent part also protects the boy. Protectors often interfere during the unburdening process because they are afraid. Since Jay has plenty of Self-energy and the boy is eager for help, the therapist promises the silent part help but is firm about asking it to wait.*

JAY: It's hesitant but willing.

THERAPIST: Great. It can check in with the boy very soon.

JAY: The boy is holding on to me so tightly now.

THERAPIST: Is that okay with you?

- *Checking on Jay's Self-energy.*

JAY: It feels great to both of us.

THERAPIST: Excellent.

JAY: We're now in my daughter's bedroom in my current house. That's where he seems to want to be.

THERAPIST: Okay with you? Now ask him if he has shown you everything he needs you to know about this experience.

- *Checking to see if witnessing can be finished.*

JAY: Yes.

THERAPIST: Is he ready to let go of his burdens?

- *After witnessing we check to see if the exile is ready to “unburden” – or let go of – toxic beliefs and extreme feeling states.*

JAY: Yes.

THERAPIST: Okay. Have him check in and around his body for all the thoughts, feelings and sensations he’s ready to let go. He can do it any way he chooses.

- *The exiled part is invited to make all the decisions.*

JAY: He wants to burn them in a bonfire.

THERAPIST: Okay. Let me know when he’s done.

JAY: It’s so freeing!

THERAPIST: Wonderful. How’s he doing?

JAY: He’s happy. He wants to play.

THERAPIST: Now let’s check with the chest pain and the silent part and anyone else who has been protecting him. Have them take a look.

JAY: They’re shocked at how happy he looks right now.

THERAPIST: Do they get that he’s safe with you? Good. What do they need?

JAY: Strangely the chest pain wants to do something physical like ride a bike. And the silent part wants to meditate.

THERAPIST: Sounds nice. Is that okay with you? Then they should do just that. Will you take care of the boy until we meet again next week by briefly checking in with him daily?

JAY: Definitely.



As we see, the distraction of somatic symptoms has protected Jay from emotional pain for many years. Effective as it can be to substitute one kind of pain for another in the short run, handling emotional pain in this way is clearly a “young” solution. Physical pain is puzzling, alarming and exhausting and does not extinguish the background noise of emotional pain. Protector actions are usually young in this way – they start young, they are cognitively young and they continue to reference the (often severely) limiting circumstances of childhood.

THE BENEFITS OF IFS FOR THERAPISTS

If you find going to work as a therapist tiring, preoccupying and overly stirring of your own fears, inadequacies and shame, IFS – in our experience – can offer relief.

- The IFS therapist is not called on to come up with the right interpretation, think up assignments, pressure clients to face the past, or put fragmented information and personalities back together again.
- We have one road map for all: access the Self. Although we know the steps that typically help us get there, our GPS is always Self-energy. When we feel lost or stumped we return to that source, tapping into our curiosity about what might be happening in this moment and calling on creativity when sessions go off the beaten path (Krause, Rosenberg & Sweezy, 2016).
- Since we view “resistance” as crucially guiding information and we don’t fight extreme protectors, the IFS approach generally evokes little negative transference (“We’re glad you showed up. What do you want us to know?”).
- The IFS focus on helping the client manifest Self-energy pulls more for connectedness than dependency.
- The non-pathologizing approach helps our clients access an open-hearted, curious, connected frame of mind. The greater their access to this wise co-therapist – the Self – the more continual their feeling of safety and comfort.

- We have the opportunity to be with our clients in the same open-hearted, curious and connected frame of mind.
- We become more self-aware by extending curiosity toward the parts of us who get triggered by our clients.
- As we differentiate from our parts and access the Self, our parallel process is the paramount invitation for the client's system to take risks and try something new.